



Use of Restrictive Practices in Non-Authorised Healthcare Settings Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group (AKG)	Non-Authorised Healthcare Settings (e.g. Emergency Departments and general hospital wards)	All workforce groups

2. Purpose

The purpose of this procedure is to outline the processes used to minimise and, where possible, eliminate the use of restrictive practices. Restrictive practices, which includes detention and/ or restraint, must only ever be used as a last resort, after exhausting all other strategies and when there is an imminent or actual health or safety risk to the individual concerned or others.

AKG acknowledges that despite every effort taken to manage patients in the least restrictive way possible, use of restrictive practices may occur. In these situations, the use of restrictive practices must be applied in a culturally respectful manner, and in consideration of the patients' rights, dignity, consent, decision making capacity and necessity.

This procedure aims to align to the policy requirements as outlined in the [Department of Health: Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#).

3. Applicability

This procedure applies to all patients within non-authorised healthcare settings such as the Emergency Department and general hospital wards/ areas. It is not applicable to the following:

- Paediatric Patients - refer to CAHS guidelines:
 - [Child and Adolescent Health Service Restraint of Patients/Clients Policy](#)
 - [Perth Children's Hospital Clinical Holding Guideline](#)
- Mental Health Patients in Authorised Mental Health Settings (Leschen) – refer to [AKG Mental Health Restraint Policy](#) and the [AKG Mental Health Seclusion Policy](#)
- Prisoners - refer to [EMHS Prisoners and Detainees: Management of Policy](#)
- Individuals who are not patients of the health service, and who are not receiving treatment, for example visitors, relatives, guardians, friends, and members of the public.
- The use of medical protective devices that protect a person's medical injury / condition. This includes but is not limited to the use of seatbelts on chairs or other devices used to support posture; or low beds and crash mats for care recipients who are a high falls risk.

4. Key definitions

Term	Definition
Behavioural Disturbance	Behaviour or psychological distress that puts the patient or others at imminent risk of serious harm and may include threatening or aggressive behaviour, extreme distress, and serious self-harm which could cause major injury or death.
De-escalation	The use of verbal and non-verbal interventions aimed at defusing anger and averting aggression.
Doctrine of Necessity	Common law doctrine that allows an individual to perform certain actions that would normally be deemed as contraventions of the law. The doctrine is only applicable in cases of imminent risk of serious harm or death to self or others.
Mechanical Restraint	The application of devices (including belts, harnesses, manacles, sheets and straps) on a person's body to restrict their movement.
Non-Authorised Healthcare Settings	Any health setting not authorised under s.541 of the <i>Mental Health Act 2014</i> . This includes Emergency Departments, general hospital wards, outpatient clinics and hospital grounds.
Physical Restraint	The application by staff members of 'hands-on' immobilisation or the physical restriction of a person to prevent the person from harming themselves or endangering others, to ensure the provision of essential medical treatment.
Restrictive Practice	Any practice or intervention that restricts an individual's rights, freedoms, or movement, including detention, mechanical, physical, or environmental restraint.
Restraint	The restriction of an individual's freedom of movement by physical or mechanical means.
Trauma informed care	A practice focused on understanding and addressing trauma's effects on a patient's physical, psychological, and interpersonal responses. Key principles include safety, trust, choice, collaboration, and empowerment.

5. Procedure

Other than in an immediate, grave emergency, there are very few circumstances in which a Health Service Provider (HSP) may lawfully restrain or detain a person in a non-authorised setting. The use of restrictive practices therefore must be reasonable and proportionate to the individual circumstances. In non-emergency situations, before commencing any restrictive practices, staff are to ensure that the following processes have been initiated:

5.1 Prevention and early recognition strategies

- Engage early with patients and carers / guardians to implement behavioural support plans, including risk screening and capacity assessment, for those at risk of challenging behaviours.
- Seek consent from substitute decision makers for those who lack capacity.

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- Initiate appropriate risk screening and assessment tools including the AKMR159.1 Behavioural Observation Form (BOF), AKMR147.6 Alcohol Withdrawal chart (CIWA-AR) and /or the AKMR27.1 Nicotine Replacement Therapy (NRT) Management and Withdrawal Chart to assess the patients' behaviours, triggers or contributing factors, levels of distress, anxiety and risk.
- Consider the patients age, cultural background, language diversity, spiritual beliefs, health literacy, past trauma, and social diversity as part of the decision-making process prior to using restrictive practices.
- Provide education to family, carers and guardians about restrictive practices and involve them in planning, reviewing and evaluating all aspects of care and support.
- Engage with Aboriginal Health Liaison Officers, Elders, and /or Traditional Healers to provide culturally sensitive patient support.
- Assess underlying medical conditions contributing to the behaviour, e.g. pain, vision or hearing impairments and mental health issues, and initiate a pharmacy review of all medications.
- Provide an appropriate physical, therapeutic and psychosocial environment, with formal and informal activities that avoid known triggers and supports prevention, care and recovery.
- Utilise de-escalation strategies such as diversional activities, sensory modulation, and a low-stimulus environment. Incorporate active listening, empathetic communication, a calm demeanour, and clear, inclusive language to reduce distress and prevent escalation.

Refer to: [Appendix 3](#) for the Minimising Restrictive Practices Infographic.

5.2 Prior to engaging the use of Restrictive Practices

In non-emergency episodes of restrictive practices, clinicians must ensure the following criteria are met prior to commencing the use of restrictive practices:

- The patient is at significant imminent risk of causing serious harm to self or others.
- All reasonable lower risk and less restrictive options have been attempted/considered, for example a low stimulus environment, single room, or a 1:1 Assistant in Nursing (AIN).
- Where practicable, the patient's consent must be obtained prior to providing any treatment, including information on all potential risks of the proposed treatment, in a manner that the patient can understand.
- Where possible, a patient's decision-making capacity must be assessed and documented by a Medical Officer to obtain informed consent for treatment. Restrictive practices must be ceased as soon as the risk is resolved, with appropriate actions taken to reassess the patient's capacity.
- If the patient lacks capacity to provide informed consent, seek authorisation from a substitute decision maker.
- Where the substitute decision maker is uncontactable, the health professional may provide urgent treatment as specified in the Guardianship and Administration Act 1990. This must be documented within the patients' medical record.
- Doctrine of necessity may be invoked only after completing and documenting a capacity and risk assessment to justify the use of restrictive practices, and/ or the patient is at significant, imminent risk of causing harm to self or others. In cases of emergency, where a formal capacity and/or risk assessment has not been completed or documented, the most

senior clinician present can authorise the use of restraint if they believe the criteria for the Doctrine of Necessity has been met, to enable a formal assessment.

- Staff should prioritise their own safety when considering the application of a restrictive practice.

Refer to: [Appendix 4](#): Fact Sheet for Common Scenarios for examples on legal considerations and proposed resolutions.

5.3 Initiation and use of restrictive practices

The following must occur once the restrictive practice has been initiated, all other attempts to de-escalate have been exhausted and the patient is presenting an imminent risk to themselves or others:

- In a planned restrictive practice event, a Medical Officer (MO) must authorise the restrictive practice including the reason for restriction, and the type of restriction used.
- However, in an emergency including a code black, the most senior clinician present can verbally authorise the use of the restrictive practice, and then follow up with the Medical Officer.
- Doctrine of Necessity – in cases of emergency, where a formal capacity and/or risk assessment has not been completed or documented, the most senior clinician present can authorise the use of restraint if they believe the criteria for the Doctrine of Necessity has been met, to enable a formal assessment per the [DOH Use of Restrictive Practices in Non-Authorised Healthcare Settings policy](#).
- The clinician may request assistance from Security staff and must, at a minimum, provide a handover outlining the risk, the rationale for restrictive practice, and the assistance required. If the clinician is unable to document this prior to initiating restrictive practice, they must complete the documentation afterward. Should Security require further clarification, they are to consult the Senior Clinician before proceeding with any interventions.
- If the restrictive practice is initiated during office hours, the shift coordinator, Associate Nurse Unit Manager or Nurse Unit Manager must be informed within 30 minutes.
- If the restrictive practice is initiated after hours, the After-Hours Registrar and After-hours Clinical Nurse Specialist (AH CNS) must be informed within 30 minutes.

5.4 High risk patient cohorts

All episodes of restrictive practice carry significant risks to patients, others and staff. A patient's response to restrictive practices will vary based on individual characteristics, the duration and intensity of the restriction, and other factors such as underlying health conditions, stress levels, trauma and the degree of aggression shown by those involved. Certain patient cohorts have higher risks associated with restrictive practices than others:

Pregnant patients

- If a pregnant patient requires restrictive practices, the patient should be placed in the left lateral tilt position to minimise the risk of aortic compression by the uterus. Pregnant patients should never be held in a face down position as this pose's risks to both the patient and the baby.
- Refer to the [Women and Newborn Health Services, Mental Health Emergency, Management of Policy](#) for further information on this patient cohort

Cognitively impaired, patients with delirium and older adults

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- Patients with cognitive impairment and/or delirium, and some older adults, are less able to understand and cooperate with medical treatment plans. Limit restrictive practices with this cohort as it may increase agitation, prolong delirium and/or increase the risk of injury. Where possible redirect the patient or ask family or carers for behavioural diversion strategies.
- Refer to the [AKG Delirium Clinical Management Guideline](#) for further information on this patient cohort.

Mechanically ventilated patients in ICU

- Restrictive practices should only be considered for patients that have been identified as a risk to themselves and others e.g. at risk of self-extubation or pulling / removing infusion lines, previous episode of violence and aggression, patients with severe psychiatric disorders, alcohol and drug withdrawal, delirium and/or patients requiring inter or intrahospital transfer.
- Refer to the [AKG Intubation and Management of the Ventilated Patient in the Intensive Care Unit Guideline](#) for further information on this patient cohort.

Serious mental illness including psychosis

- There may be an increased risk of harm to patient and staff when using restrictive practices to intervene with a patient experiencing symptoms of serious mental illness or psychosis because of their perceived fear or confusion.
- Refer to the [AKG Mental Health Consultation Liaison Service Procedure](#)
- General wards are not authorised under the Mental Health Act 2014 (MH Act 2014) and therefore the provisions for restraint and seclusion in the MH Act 2014 do not apply in the general ward settings, as such this procedure is to be used.

Alcohol and other drugs

- Patients who use alcohol and other drugs may be at a higher risk of behavioural disturbances. Identify and manage imminent risks and commence appropriate withdrawal charts such as the AKMR147.6 Alcohol Withdrawal chart (CIWA-AR) or the AKMR27.1 Nicotine Replacement Therapy (NRT) Management and Withdrawal Chart. Refer to the Mental Health Consultation Liaison Service - Alcohol and Other Drugs Clinical Nurse Specialist for patients suspected of having alcohol or other drug risks.
- Refer to the AKG [Alcohol and Other Drugs – Patient Management Guideline](#) for further information on this patient cohort.

Where there is a risk of harm to staff, patient's or visitors, staff are to request Hospital Security assistance by dialling **55** to initiate a **Code Black**.

5.5 During the restrictive practice – patient care management

Restrictive practices must be reasonable and proportionate to the circumstances, re-assessed regularly and removed/reduced as soon as the level of risk/harm has diminished. Any deterioration in a patient's physical condition, mental state or cognitive state must be assessed and managed promptly in accordance with [MP 0171/22 Recognising and Responding to Acute Deterioration Policy](#).

A Restraint Lead and/ or Senior Clinician should be identified and take responsibility for leading the team through the restrictive practice. **Refer to: Table 1, Appendix 1** for a comprehensive list of clinician roles and responsibilities.

During the restrictive practice, the Restraint Lead is responsible for patient safety and ensuring the patients dignity and respect is maintained, whilst considering previous trauma and providing culturally safe care. **Refer to: Table 2, Appendix 2** for a comprehensive list of restrictive practice positions and their associated risks.

Always:

- When repositioning a patient from a standing position to a prone (face-down) or supine (face-up) position on the floor, the movement must be carried out in a controlled manner. Staff must always prioritise their own safety when repositioning a patient. Assistance may include supporting the patient's arms and, if necessary, protecting the patient's head.
- In instances where prone restraint exceeds three minutes, the Restraint Lead must be actively involved in planning the safest approach, and where possible, the restraint should be released to reposition the individual; if repositioning is not appropriate at the three-minute mark, the rationale and actions taken must be clearly documented in the patient medical record.
- Do not carry / drag the patient, use safer options such as a wheelchair if the patient is declining to ambulate.
- Whenever a restrictive practice is required, a face-up (supine) position should be used where it is safe to do so.
- A transport hold is encouraged to walk patients to a private area or room for de-escalation.
- Once a restrictive practice has begun, staff must check and document on the patients AKMR140.3 Adult Observation and Response Chart (AORC) within 15 minutes from commencement of the restraint, and then as a minimum hourly thereafter or as directed by the Medical Officer.
- Continuously monitor the patient during the restrictive practice and disengage when safe to do so.

Never:

- Apply direct pressure to the neck, trachea, chest, abdomen, back, or pelvic regions.
- Use head locks, choke holds, or similar techniques.

Avoid:

- Occluding the nose and mouth unless medically indicated, e.g. use of an oxygen mask.
- Using restrictive practices that prevents the patient from communicating.
- Avoid pulling, rotating, or flexing the neck in any direction.
- Avoid hyperflexion of the trunk in standing or seated restraints.

If concerns regarding the airway are raised, the restrictive practice must stop immediately.

If a patient physically deteriorates or reports being unable to breathe, an A-E physiological assessment must occur and identified issues addressed.

Dial 55 to activate procedures for a **MET call and Code Blue** in the event of a **medical emergency**.

5.6 Cessation of restrictive practices

The use of a restrictive practice must cease once the episode is complete and/or when the risk to the patient or others has returned to an acceptable level. A decision to cease using a restrictive practice must be:

- Based on clinical judgement.
- Informed by the ongoing assessment of the patient's mental and physical state, their capacity, their ability to control their behaviour and the subsequent level of imminent risk/harm to themselves and others.

5.7 Recovery after restrictive practices

Patient:

All patients require careful management and constant monitoring of their physical and mental health after the restrictive practice is ceased.

- Complete a full set of physiological observations, document on the AKMR140.3 Adult Observation & Response Chart (AORC) and escalate according to the score. If the patient refuses, document this in the patients' medical record.
- As directed by the [DOH Use of Restrictive Practice Policy](#), any patient who meets any of the following criteria must have a post restrictive practice medical review:
 - Any restrictive practice where patient or staff are harmed or experience trauma.
 - Any restrictive practice where the patient is prone for more than 3 minutes.
 - Any restrictive practice resulting in a clinical incident.
- Where appropriate, open disclosure should be considered with patients and /or carers to determine the appropriateness of the intervention. When the decision is made to not debrief with the patient and/or guardian, the reasons for not debriefing must be documented in the patient medical record, in accordance with the [DOH Use of Restrictive Practice Policy](#).
- Update patient alerts on relevant systems to inform all staff of patients assessed as having a significant risk of aggressive behaviour. Where appropriate add alerts to:
 - Emergency Department Information System (EDIS)
 - Psychiatric Services Online Information System (PSOLIS)
 - Clinical Manager and SHIMS as Patient of Concern (POC)
 - webPAS Aggression Alerts
 - SENTRY Hospital Security Database
 - Enterprise Bed Management (EBM)
 - Daily Hospital After-Hours Team (HAT) Summary.
- Clinical handover is crucial for harm minimisation. It must cover recent capacity and risk assessments, behavioural changes, potentially escalating behaviours, and existing care plans and tools in place like the BOF.
- Communicate patients of concern to all staff who may have contact with that patient. This includes Patient Support Services and Security staff who do not attend clinical handover.

Staff:

- Provide first aid for staff who have may have been injured.

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- Where practicable and appropriate, staff debriefing sessions are to be undertaken as soon as possible and in accordance with the EMHS Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy.
- Assess the work environment and implement additional safety measures as needed. Seek guidance and support from Work Health and Safety as appropriate.
- Work Health and Safety are available to assist and support debriefing sessions. Staff members may seek individual counselling through the [Employee Assistance Program \(EAP\)](#) without the need to inform a manager or supervisor. All sessions are completely confidential.

6. Documentation

- Every event of restrictive practice will be documented on the appropriate form e.g. AKMR 159.4 Physical Restraint Record. Documentation must include the following information:
- Details of the situation, including the patient's mental state, deterioration, associated behaviours and the level and type(s) of risk.
- Unsuccessful lower risk de-escalation strategies utilised and reasons for failure, if applicable.
- The type of restrictive practice used and clinical rationale.
- Assessment of mental capacity and the outcome of that assessment.
- Confirmation that consent to treatment was sought and the outcome.
- The use of supplementary support from security officers, if applicable.
- Any physical injuries resulting from the use of a restrictive practice.
- Notification of family member, guardian, enduring guardian or substitute decision-maker, if appropriate.
- Any medications offered or administered.
- Any multidisciplinary reviews conducted.
- Any relevant code black report(s).
- Any restraint monitoring tool(s).
- On cessation of the restrictive practice, the BOF should be updated, commenced if not already in place, completed at a minimum of hourly, and any interventions documented.
- Post restrictive practice medical review must be documented in the patient medical record.
- Physiological observations must be documented using the appropriate AORC.
- Post restrictive practice events require a risk re-assessment in the patients AKMR119 Comprehensive Care Plan and if required, updating the relevant nursing care interventions.
- If a non-registered companion / assistant in nursing is observing the behaviourally disturbed patient, the BOF must be reviewed by the primary nurse throughout the shift.
- In the event of a patient being harmed, a Datix CIMS must be completed as per the [EMHS Post Incident Management / Critical Incident Stress Debriefing Policy](#).
In the event of a staff member being harmed, a CHoIR form must be completed in accordance with the [EMHS Prevention and Management of Workplace Violence and Aggression Policy](#).

7. Compliance monitoring

Compliance and reporting against this procedure will be the responsibility of the Comprehensive Care Steering Committee.

The [DOH Use of Restrictive Practices in Non-Authorised Healthcare Settings policy](#) requires an annual report to be completed by the 31st July each year detailing the following:

- Established processes are in place to ensure the documentation for each use of restrictive practice in a non-authorised setting.
- Rates of restrictive practices applied in non-authorised healthcare settings, including the type of restrictive practice used.
- Rates of clinical incidents resulting from the use of a restrictive practice. This includes reporting in accordance with MP 0122/19 [Clinical Incident Management Policy](#).
- Governance and decision matrixes used to assess a patient's decision-making capacity each time a restrictive practice is utilised.

AKG will comply with this requirement and complete reports bi-annually to strengthen compliance monitoring and improve KPI performance.

8. Supporting information

The following documents support the implementation of this procedure:

- [Armadale Health Service Emergency Response Plan Code Black](#)
- [AKG Alcohol and Other Drugs – Patient Management Guideline](#)
- [AKG Delirium Clinical Management Guideline](#)
- [AKG Intubation and Management of the Ventilated Patient in the Intensive Care Unit Guideline](#)
- [AKG Mental Health Consultation Liaison Service Procedure](#)
- [AKG Mental Health Restraint Policy](#)
- [AKG Recognition and Response to Acute Deterioration Policy](#)
- [AKG Safe Use of Bedrails Guideline](#)
- [EMHS Discharge Against Medical Advice Policy](#)
- [EMHS Missing or Suspected Missing In-Patients – General Policy](#)
- [EMHS Open Disclosure Policy](#)
- [EMHS Post Incident Management / Critical Incident Stress Debriefing Policy](#)
- [EMHS Prevention and Management of Workplace Violence and Aggression Policy](#)
- [EMHS WHS: Incident and Hazard Reporting Policy](#)

9. Legislative and policy context

The following documents are required to give effect to this procedure:

- [Office of the Chief Psychiatrist Use of Detention and Restraint in Non-Authorised Healthcare Settings Guideline](#)
- [HDWA Clinical Incident Management Policy](#)
- [HDWA Recognising and Responding to Acute Deterioration Policy MP0171/22](#)
- [HDWA Refusal or Withdrawal of Care for a Patient Exhibiting Aggressive or Violent Behaviour Policy MP 0174/22](#)
- [HDWA Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy MP 0186/24](#)
- [Western Australia: Guardianship and Administration Act 1990](#)

- [Western Australia: Work Health and Safety Act 2020](#)

10. National standards

[National Safety and Quality Health Service \(NSQHS\) Standards \(second edition\):](#)

- Clinical Governance Standard: 1.1, 1.2, 1.6,
- Partnering with Consumers Standard: 2.6, 2.7, 2.10,
- Medication Safety Standard: 4.2, 4.3, 4.4, 4.10, 4.15
- Comprehensive Care Standard: 5.35
- Communicating for Safety Standard: 6.2, 6.3, 6.4, 6.7, 6.8, 6.9, 6.10, 6.11
- Recognising and responding to acute deterioration 8.03, 8.04, 8.05, 8.06, 8.10.

11. References

1. [Australian Commission on Safety and Quality in Health Care. \(2021\). Minimising Patient Harm.](#)
2. [Chief Psychiatrist of Western Australia, \(2023\) Restrictive Practices Fact Sheet.](#)
3. Department of Health, (2016). Aggression Prevention & Intervention (API). Instructor Guide: Restrictive Interventions, Practical Skills Manual.
4. Royal Perth Bentley Group, (2023) [Safe Use and Management of Patient Restraints CPS.](#)
5. [Women and Newborn Health Services, \(2020\) Mental Health Emergency, Management of. Guideline](#)

12. Document Owner

Enquiries relating to this document may be directed to:

Title: Service Director Mental Health/ Comprehensive Care Executive Sponsor
Department: Executive Team
Email: Karen.Elliott@health.wa.gov.au

13. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
AK0000/22	xx/04/2025	dd/mm/yyyy	

14. Authorisation

This procedure has been authorised and issued by:

Authorised by	
Authorisation date	dd/mm/yyyy
Published date	dd/mm/yyyy

Appendix 1: Table 1: Roles and responsibilities

Role	Responsibility
Restraint Lead (Senior Nurse/Medical Officer)	- Assign response team members. - Maintain the safety of the environment and all individuals present. - Record and document restraint episodes or delegate task. - Ensure appropriate documentation is completed if the patient is placed under the Mental Health Act 2014. - Inform NUM / AH CNS of restraint incident. - Manage and monitor the patient's airway. - Support and protect the head and neck, ensuring unrestricted movement to avoid adverse risks. - Maintain verbal communication with the patient and de-escalate when possible. - Monitor airway, breathing, and circulation and that the patients' limbs are not being injured. - Inform the lead clinician of the time frames the patient has been restrained in high-risk positions. - Communicate with and instruct the response team. - Monitor the interventions of other team members. - Communicate observations to the rest of the team for decision-making purposes. - Conduct a post-restraint debrief with response team members.
Team members	- Follow Restrain Lead instructions. - Maintain airway management. - Restraint of arms and legs. - Ongoing assessment of patient's clinical condition and response to restraint- provide feedback to Restraint Lead. - Administration of medications as prescribed.
Security staff	- Receive iSoBAR handover from clinical staff on arrival, including any medical conditions or considerations affecting patient safety during restraint, and the rationale for intervention. - Restraint of arms and legs. - Security staff will follow the guidance of senior clinicians when making decisions related to patient restrictive practices.
Scribe/Timer	Document events that occur during the restraint including: - Psychological interventions (e.g. de-escalation attempted). - Medical Intervention (e.g. physical examination). - Medication administered.

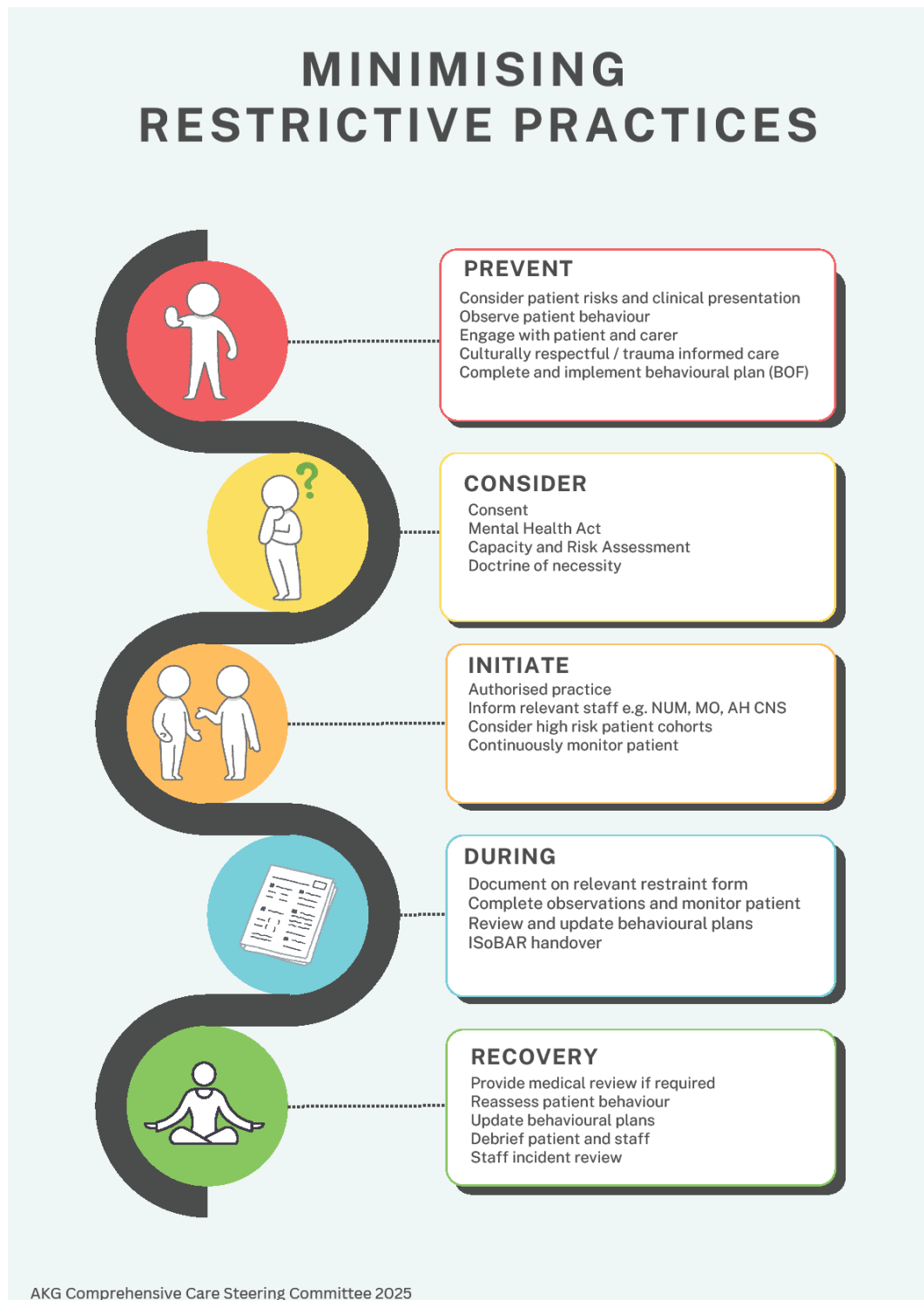
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Role	Responsibility
	• Type / time of restraint e.g. (Physical restraint commenced at 1400). • Start / finish time if moved into prone position. • If prone for >3 minutes, MUST document reasonings and actions. • Change of staff members – care of head, left arm, right arm, legs. • Physiological monitoring and findings. • Any injury or harm to patient / staff / others.

Appendix 2: Table 2: Restrictive practices positions and their associated risks

Position	Risks	Risk reduction strategies
Standing	• Risk of asphyxiation. • Choking.	• No pressure placed on the head, neck or back. • Avoid hyperflexion of neck.
Kneeling	• Musculo-skeletal injury. • Positional asphyxia.	• No pressure placed on the head, neck or back. • Avoid hyperflexion of neck.
Prone (face down)	• Sudden unexpected deaths have been reported in this position. • May restrict the ability to breathe lessening the supply of oxygen to meet the body's needs. • Should only be used as an intervention of last resort.	• Continual patient monitoring and communication. • No pressure placed on the head, neck or back. • Patient not to be placed in prone maximal restraint position (hogtied). • Should be limited to the minimum amount of time necessary to change position preferably kneeling or standing i.e. < 3 minutes.
Supine (face up)	• Choking or inhalation of vomit and aspiration. • Risk of asphyxia, fatal cardiac arrhythmia or respiratory distress.	• Keep patient's head or bed elevated to reduce aspiration risk.
Seated	• Leaning forward during prolonged restraint increases the risk of harm or death, especially with a higher body mass index. • Wrapping patients' arms around their front and holding from behind can increase the risk of asphyxia.	• Patient to be seated upright. • Avoid hyperflexion of the spine. • Do not wrap patients' arms around the front of their body.

Appendix 3: Minimising Restrictive Practices Infographic



Appendix 4: Fact sheet for Common Scenarios – Involving Restraint in Non-Authorised Settings:

Hypothetical Situation	Legal Consideration	Proposed Resolution
A patient is brought into the ED by a community clinician under the MHA Form 1A to MHU. The patient initially agrees to stay in the ED while awaiting a bed in the MHU but later changes their mind and refuses to stay. A Form 3A has been completed to detain the patient. Can a clinician request security staff to restrain the patient under the MHA when the patient attempts to leave?	Mental Health Act considerations Form 1A = Referral Order for examination by psychiatrist at MHU. Patient cannot be restrained unless they are received at MHU. Form 3A = Detention Order Patient can only be 'detained' but NOT restrained. Doctrine of Necessity If patient is at risk of experiencing serious harm to themselves or others (ie. they are threatening to kill themselves or others), then they can be restrained under Doctrine of Necessity.	Patient can only be detained but NOT restrained at ED if they are Formed to MHU. Direction to security officers: 1) clinicians cannot direct them to 'restrain' the patient if there are no imminent risk of self-harm or harm to others. 2) If patient is demonstrating imminent risk of self-harm or harm to others, then the clinician can direct security officers to assist by temporarily restraining the patient to prevent the serious risk of harm occurring, which would be lawfully justified under Doctrine of Necessity. Any use of force to restrain the patient must be minimum force necessary.
A patient is brought in by WAPOL and the CMH team after being placed on the MHA from 1A. The MHA Form 1A is addressed to the ED and accepted by the ED clinician. The patient is refusing to stay in the ED and poses a high risk to themselves and others. Can a clinician request security staff to restrain the patient when the patient attempts to leave?	Mental Health Act consideration If patient is 'Formed' to ED and patient is 'received' in ED, then reasonable force may be used to detain and if necessary, restrain the patient to stop them from leaving. Doctrine of Necessity If patient is at risk of experiencing serious harm to themselves or others (ie. they are threatening to kill themselves or others and this is considered to be a 'real' risk), then they can be restrained under Doctrine of Necessity.	Patient can be temporarily 'restrained' in ED only if they are on a Form 1A and the place of examination is written as Armadale Hospital ED. Direction to security officers 1) Clinicians can direct security officers to temporarily restrain patient under MH Act (preferred option); Or 2) If patient is demonstrating imminent risk of self-harm or harm to others, then the clinician can direct security officers to assist by temporarily restraining the patient to prevent the serious risk of harm occurring, which would be lawfully justified under Doctrine of Necessity. Any use of force to restrain the patient must be the minimum force necessary.
A patient is accepted into the ED on Form 1A, reviewed by a consultant, and placed on MHA 3D. Form 3A is also in place. The patient refuses to stay, displaying no self-harm thoughts but presenting in a confused state. Can a clinician request security staff to restrain the	Mental Health Act consideration Form 3A = pt can be detained for 24 hrs for examination but CANNOT be restrained. Form 3D = pt is waiting to be transferred to the authorised unit. It does not appear that pt can be restrained until received at authorised unit. Doctrine of Necessity	Capacity assessment Clinician to determine if patient has decision making capacity and understands the risk of leaving the hospital. • If yes, then the clinician cannot direct security officers to restrain patient. • If no, clinician needs to exercise clinical judgement to determine if it is necessary to detain/restrain the patient to prevent serious injury/harm to the patient.

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patient when the patient attempts to leave?	Can only justify the restraint of patient to prevent imminent threat of serious harm/danger to patient or others. Capacity issue Consider whether patient has capacity to understand the risk of leaving the hospital. If patient lacks capacity, clinical judgement required as to whether they will be at risk of serious harm if they left the hospital.	Direction to security officers 1) if there is an imminent risk of harm to patient or others, clinician can direct security officers to restrain the patient (as a last resort); Any use of force to restrain the patient must be minimum force necessary.
A patient is brought in by SJA, presenting as intoxicated, aggressive, and confused. They have no mental health history and do not meet the criteria for placement under the MHA. The patient wishes to leave the ED. Is there any basis to detain and restrain the patient from any harm?	Mental Health Act consideration Not applicable Capacity issue Consider whether patient has capacity to understand the risk of leaving the hospital. If yes, and there is no imminent risk of serious harm/threat to themselves/others, then Doctrine of Necessity does NOT apply. No lawful ability to detain/restrain. If pt lacks capacity and poses imminent risk of serious harm/threat to themselves/others, then Doctrine of Necessity may apply.	Capacity assessment Clinician to determine if patient has decision making capacity and understands the risk of leaving the hospital. - If yes, then the clinician cannot direct security officers to restrain patient. - If no, clinician needs to exercise clinical judgement to determine if it is necessary to detain/restrain the patient to prevent serious injury/harm to the patient. Direction to security officers 1) if there is an imminent risk of harm to patient or others, clinician can direct security officers to restrain the patient (as a last resort). Any use of force to restrain the patient must be minimum force necessary.
A patient with a history of autism and an eating disorder refuses to stay in the medical ward and declines treatment, requesting to leave the hospital. The patient is at risk of medical complications if treatment is not received. Can the clinician direct restraint and enforce medical treatment?	For patient who is under 18 years of age or lacks capacity: 1) Check if pt has been appointed guardian to make treatment decisions for them. If yes, seek consent from guardian to agree to treatment. 2) If medical complication is life threatening and pt lacks capacity to understand this risk, and we are unable to locate guardian to make the decision to consent to treatment; then clinician can provide urgent treatment under the <i>Guardianship & Administration Act 1990</i> 3) Can seek guidance from the Office of Public Advocate in relation to guardianship issues Doctrine of Necessity Can only justify the use of restraint of patient to prevent imminent threat of serious harm/danger to patient or others. Clinician needs to determine how serious is the risk of medical complications if treatment is not received.	MH Act assessment Does patient fall under the criteria of being formed under MH Act? If yes, pt can be restrained under MH Act. If pt is not under MH Act Determine if there is medical concerns/risk to see if there's justification to detain/restrain the person for the risk of serious harm under Doctrine of Necessity. Capacity assessment Clinician to determine if patient has decision making capacity and understands the risk of leaving the hospital. - If yes, then the clinician cannot direct security officers to restrain patient. - If no, clinician needs to exercise clinical judgement to determine if it is necessary to detain/restrain the patient to prevent serious injury/harm to the patient. Enforcing treatment

Restrictive Practices in Non-Authorised Healthcare Settings Procedure

	If there is imminent risk of harm to patient if treatment is not received, then doctrine of necessity may apply.	Consider guidelines under WA Eating Disorders Outreach & Consultation Service (WAEDOCS) WAEDOCS Eating Disorders Management of Youth Adults update July 2020 (health.wa.gov.au) RPBG Eating Disorders: Management of Patients SOP. All medical decision should be made by consultants.
A patient under the MHA refuses to stay in the ED, and the clinician requests security staff to prevent the patient from leaving the hospital. However, the security staff refuses to comply. Should the security staff follow the clinician's direction?	Is patient 'Formed to ED' under Form 1A? If so, patient is then received at place of examination, then patient can be detained/restrained. Does Doctrine of Necessity apply? - Is patient in imminent risk of serious harm to themselves/others - If yes, Doctrine of Necessity applies & patient can be restrained. - If No, patient cannot be restrained.	The Security officer should follow the Clinician's directions. If the security officer is challenging the clinician's judgement as to whether it is legal to restrain the pt, the clinician needs to escalate this to the Security Officer's Manager (eg. Shane Ball) and document this incident. If the security officer does not follow the clinician's direction due to safety concerns for themselves, they need to inform the clinician and consider alternative options – eg. call police if appropriate.
A patient on Form 6B (Inpatient treatment order) in the medical ward was reviewed by a consultant and advised to receive ECT for catatonic schizophrenia. The patient's NOK obstructed the nursing staff from taking the patient to the theatre for ECT, resulting in the patient not receiving the treatment. Could Form 12C be applicable in non-authorized areas? Can security be used to remove the NOK who is obstructing the treatment?	ECT can be provided to adult voluntary patients with their consent or if they are unable to give informed consent, with the consent of someone who is allowed to make decisions for them. If patient is able to give informed consent and has consented to receiving ECT, the NoK cannot obstruct and prevent the treatment. If NoK is obstructing treatment, security can be called to assist. If a psychiatrist is satisfied that it is in the best interest of the patient, Form 12C can be used to make an order under section 262 of the MH Act to prohibit or limiting the patient communicating with people, including visitors from mental health advocate or their lawyer at all reasonable times. The Order can also prohibit/limit the patient from sending/receiving phone calls or emails. The MH Act does not expressly say that Form 12C can only be used in authorised settings. Thus, Form 12C can be used if a person is under an inpatient treatment order (eg. Form 6B) in a non-authorized area of the hospital.	MH Act assessment Can patient give informed consent to ECT? If yes, NOK cannot interfere/obstruct with treatment decision. Can Form 12C be used in non-authorized setting to restrict communication from patient to NOK If a psychiatrist considers it is in best interest of patient to restrict their communication, Form 12C can be used in non-authorized setting. Can security officer be called to assist? If required, and situation cannot be de-escalated by clinical staff, security officers can be called to assist to escort the NoK off the premises if they are being aggressive and disruptive to the hospital. Security officers have the authority to remove the 'unwanted person' with minimum force necessary. Any use of force by security must be lawful, justifiable and proportionate. Prior to consider the use of force, a Security officer should first attempt to persuade the person to leave.